BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones show order, not equal time intervals or booked appointments. The treating team requests tests, reviews results and confirms treatment. full blood count (FBC), renal and liver profile before each treatment, including each weekly dose. Review neuropathy, function, infusion reactions and symptoms before every administration. Default Days 1, 8, 15 and 22 every 28 days. The consultant may choose Days 1, 8 and 15 only; confirm the selected plan. Each dose needs its own review.

Check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Blood / kidney / liver	Baseline FBC, kidney and liver tests	FBC, kidney and liver tests before each weekly treatment	FBC, kidney and liver tests before each weekly treatment	Recheck recovery and dose before restarting	Follow unresolved abnormalities
Symptoms / medicines	Review symptoms, medicines and previous treatment	Review effects before each dose	Continue symptom and medicine reviews	Report problems; do not change or restart doses yourself	Agree recovery and next treatment plan
Heart / electrocardiogram (ECG) / echo	Review cardiac symptoms and monitor observations during infusion. No fixed routine echo schedule is imposed for this paclitaxel monotherapy; significant conduction abnormalities require cardiac monitoring.	Review symptoms and indicated tests	Continue the risk- and treatment-specific plan	Urgent assessment for chest pain, fainting or breathlessness	Agree follow-up from exposure and results
Response / course	Confirm cancer-specific treatment aim	Continue the agreed weekly pattern while helpful and tolerated	Review benefit, symptoms and the agreed scan plan	Reassess if treatment is held or symptoms change	Agree continuation, change or completion
Bone health / other care	Review previous/ongoing treatments and fracture risk	No automatic chemotherapy-cycle dual-energy X-ray absorptiometry (bone density) (DXA)	Keep any separate endocrine/ fracture-risk plan	Review if clinical risks change	Continue separately indicated follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00226 v9 · official regimen · HPRA conventional paclitaxel product information; 09 January 2026

THE WHOLE TREATMENT JOURNEY

Weekly paclitaxel: treatment and checks

Shared advanced-cancer protocol

Paclitaxel on Days 1, 8, 15 and 22 of a 28-day cycle; a Days 1, 8 and 15 schedule may be selected by the consultant. Confirm your appointment plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00226	Confirm indication, goal and current prescription	Continue the agreed weekly pattern while helpful and tolerated	Continue according to response, tolerance and prescribed course	Team review before restart or dose change	Agree completion or next treatment
Blood / kidney / liver NCCP 00226 · SMPC	Baseline FBC, kidney and liver tests	FBC, kidney and liver tests before each weekly treatment	FBC, kidney and liver tests before each weekly treatment	Recheck recovery and dose before restarting	Follow unresolved abnormalities
Symptoms / medicines NCCP 00226 · SMPC	Review symptoms, medicines and previous treatment	Review effects before each dose	Continue symptom and medicine reviews	Report problems; do not change or restart doses yourself	Agree recovery and next treatment plan
Heart / ECG / echo NCCP 00226 · SMPC	Review cardiac symptoms and monitor observations during infusion. No fixed routine echo schedule is imposed for this paclitaxel monotherapy; significant conduction abnormalities require cardiac monitoring.	Review symptoms and indicated tests	Continue the risk- and treatment-specific plan	Urgent assessment for chest pain, fainting or breathlessness	Agree follow-up from exposure and results
Response / course NCCP 00226 · SMPC	Confirm cancer-specific treatment aim	Continue the agreed weekly pattern while helpful and tolerated	Review benefit, symptoms and the agreed scan plan	Reassess if treatment is held or symptoms change	Agree continuation, change or completion
Bone health / other care NCCP 00226 · SMPC	Review previous/ongoing treatments and fracture risk	No automatic chemotherapy-cycle DXA	Keep any separate endocrine/ fracture-risk plan	Review if clinical risks change	Continue separately indicated follow-up

What each check is for

1 Treatment and safety review

Before each planned treatment or four-week review for olaparib

FBC, renal and liver profile before each treatment, including each weekly dose. Review neuropathy, function, infusion reactions and symptoms before every administration.

Ask / **act:** The treating team confirms results and the plan; this map does not authorise a dose.

Sources: NCCP 00226 · SMPC

2 Benefit and continuation

At agreed clinical review points

Weekly paclitaxel can shrink or slow metastatic breast cancer. In a breast cancer study it produced more tumour responses than treatment given less often, but more severe nerve symptoms. Some trial participants also received trastuzumab, so the result does not predict every monotherapy outcome.

Ask / **act:** Discuss benefit, toxicity and priorities before continuing or changing treatment.

Sources: NCCP 00226 · RETROSPECTIVE-TRIAL

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.21